

Not All Dementia Care is Equal: A Decade of Medicare Spending Across Eight States

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Background and Motivation

We know dementia is expensive. Medicare spends roughly \$10,000 more per person on patients with dementia (PWD) than those without, every year, across every state. In 2025, the total economic burden of dementia will exceed \$780 billion nationally, with Medicare accounting for \$106 billion of that cost alone (USC Schaeffer Center, 2025). For dementia patients, spending has long been dominated by institutional care, which is costly and not always what patients or families prefer (Alzheimer's Association, 2024)

Research Questions and Hypotheses

How does the composition of Medicare spending differ across states for patients with dementia (PWD) vs. patients without dementia (PWoD), and has that service-category mix shifted between 2011 and 2021? *We expect states such as California, New York, and Illinois to show higher per-capita Medicare spending and a greater share of spending on post-acute services compared to states such as Mississippi and Georgia, due to differences in care delivery patterns, service mix, and underlying cost structures. We further expect these differences to persist, and potentially widen, over the decade examined.*

Data and Methods

- Data:** Medicare Fee-for-Service claims, aggregated to state-year-patient group level Years: 2011, 2021 States: California, New York, Texas, Florida, Illinois, Ohio, Georgia, Mississippi Groups: PWD (patients with dementia) vs. PWoD (patients without dementia). The PWoD population is adjusted for age, sex, and race to match the dementia population, the only difference between groups is dementia status. Dollar values: Inflation-adjusted to 2021 dollars
- Service categories:** Inpatient Hospital Care, Skilled Nursing Facility Care, Home Health Services, Hospice Care, Outpatient Services, Carrier/Physician Costs, Durable Medical Equipment, Prescription Drugs, Medicare Part D
- Approach:** Descriptive per-capita spending analysis using stacked bar charts to reveal how the cost mix evolves over time and varies across states

Results

- In 2011, Mississippi showed the largest per-capita spending gap between PWD and PWoD (~\$11,000), while Georgia and Ohio showed the smallest gaps (~\$4,500–\$5,000). By 2021, the gap in Mississippi dropped sharply to ~\$4,500, while New York's gap grew to ~\$11,000, the largest of all 8 states that year.
- In 2011, Inpatient Hospital Care and Skilled Nursing Facility costs account for the largest shares of PWD spending across all 8 states. California and New York show the highest total per-capita spending (~\$40,000–\$45,000), while Mississippi and Georgia show the lowest (~\$30,000–\$35,000).
- Between 2011 and 2021, total PWD spending increased across most states, most visibly in California and New York. Inpatient and SNF costs remained the largest categories in both years, though their relative shares declined modestly in several states. Carrier Costs (doctor visits) and Prescription Drug (Part D) spending grew as a share of total spending across most states by 2021.

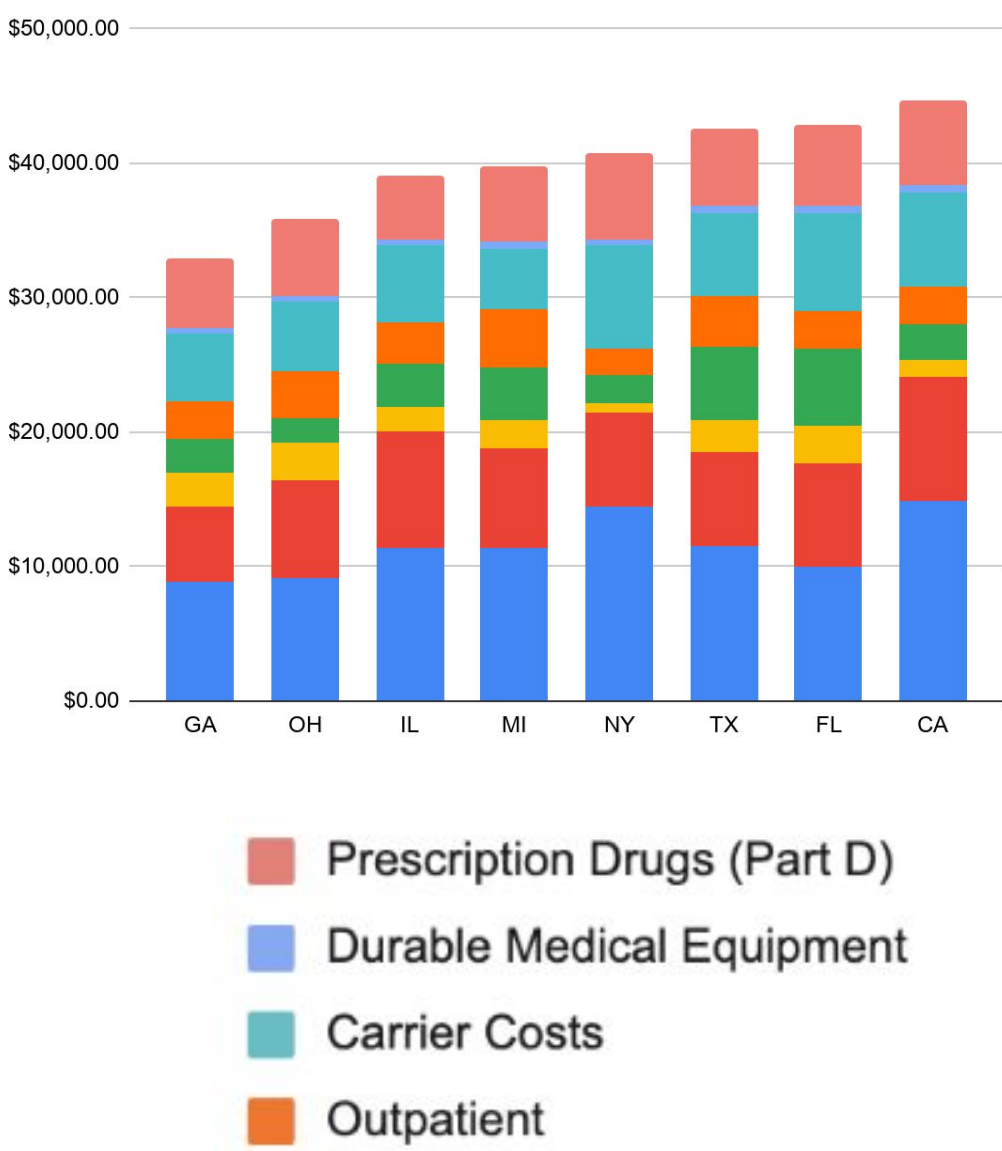
Conclusions

- The per-capita spending gap between PWD and PWoD is substantial across all 8 states, but shifted over the decade, Mississippi's gap narrowed considerably while New York's widened, suggesting these differences are not static.
- Inpatient and SNF costs remain the dominant spending categories for PWD in both years, though Carrier and Part D spending grew across most states by 2021, indicating a gradual shift in service mix.
- Differences in spending levels across states likely reflect a mix of medical service costs, care delivery patterns, and the broader state policy environment, higher spending does not necessarily mean higher utilization.
- These patterns will likely grow more consequential as the older adult population expands, particularly in Southern states.

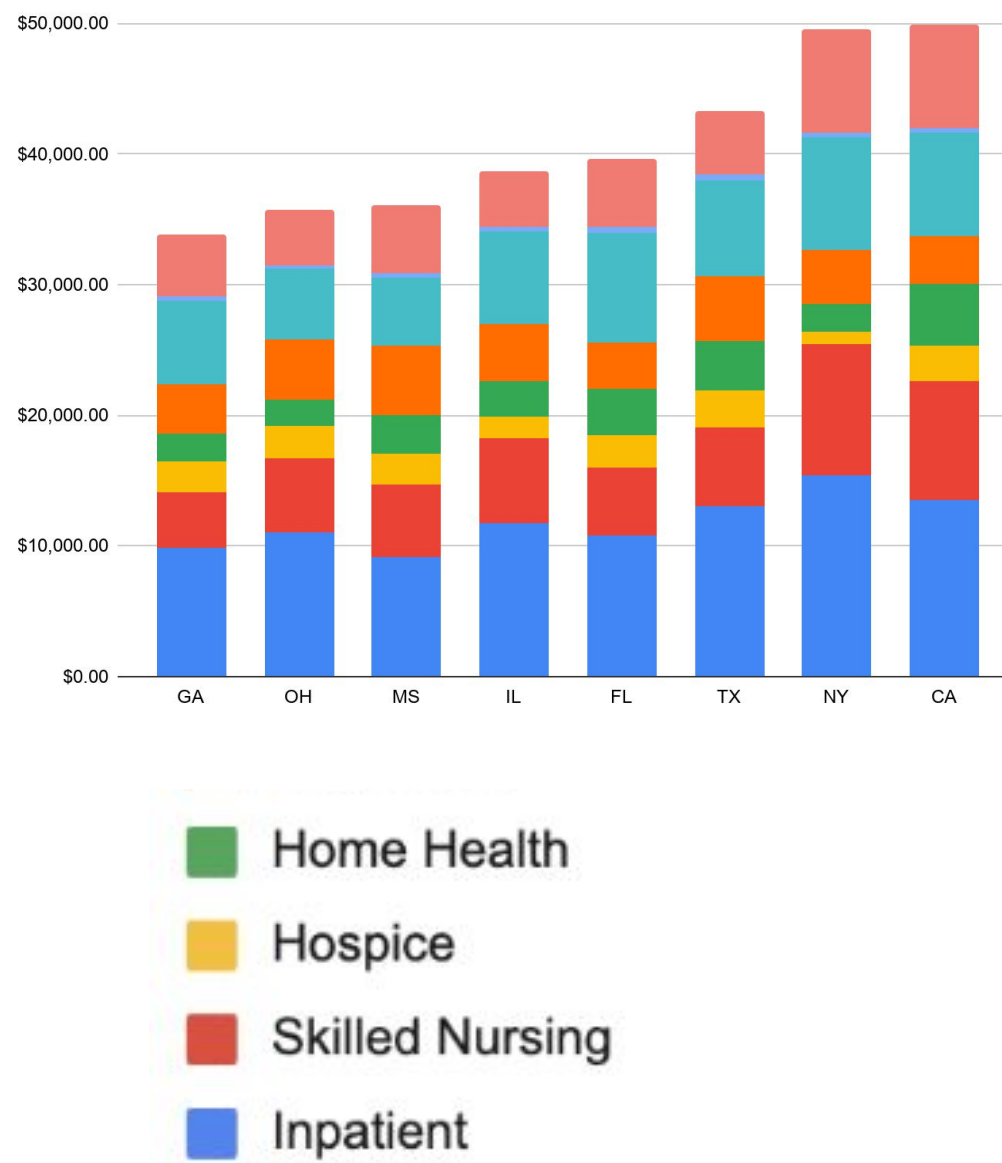
Limitations

- Medicare Fee-for-Service claims only; excludes Medicare Advantage beneficiaries (~50% by 2021)
- Three benchmark years may miss intermediate policy changes
- Higher costs across states may partly reflect prices rather than utilization
- Cannot observe informal caregiving, Medicaid co-payments, or dual-eligible spending allocation

Medicare Cost Category Breakdown (2011) (Pw/D)



Medicare Cost Category Breakdown (2021) (Pw/D)



Net Medicare Cost Difference for PwD vs. Pw/oD (2011 - 2021)

